Whitaker Physical Therapy

0607 Michelle Ports
Conroe, TX
Phone 346-555-6416

Itemized Medical Bill

Patient:	Melanie Chang
Date of birth:	21 Nov 1981
Insurance claim:	C-1477
Date of injury:	28 Feb 2025
First visit:	28 Feb 2025
Last visit:	6 Jun 2025
Number of visits:	17

Services

Service	Units	Charge
99203 New patient evaluation	1	$339.99
98941 Spinal manipulation	7	$785.34
97110 Therapeutic exercise	2	$342.27
97124 Massage therapy	3	$305.57
72040 X-ray, cervical spine	1	$1,157.55
20552 Trigger point injection	3	$1,370.02
TOTAL		$4,300.74

Recent visits

Date	Service	Charge
8 May 2025	97110 Therapeutic exercise	$154.53
15 May 2025	98941 Spinal manipulation	$116.90
19 May 2025	72040 X-ray, cervical spine	$1,157.55
25 May 2025	20552 Trigger point injection	$659.96
30 May 2025	20552 Trigger point injection	$350.78
6 Jun 2025	98941 Spinal manipulation	$104.41

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Treatment History and Summary

Patient:	Melanie Chang
Treating provider:	Barbara Harrell, DC
Diagnosis:	concussion

Treatment history by month

Month and services	Visits	Charges
February 2025: evaluation	1	$339.99
March 2025: chiropractic adjustment, pain management, physical therapy	4	$1,013.48
April 2025: chiropractic adjustment, massage therapy	3	$322.25
May 2025: chiropractic adjustment, imaging, massage therapy, pain management, physical therapy	6	$2,090.61
June 2025: chiropractic adjustment	1	$104.41

Treatment summary

Patient persented following a low-speed collision in a parking lot with reoprted head impact and subsequent headache, dizziness, and difficulty concentrating. Over the course of 17 visits, the patient received care consistent with standard management of concussion, including symptom monitoring, vestibular and cognitive assessment, graduated activity guidance, and gradual return-to-activity planning. The mechanism of injury is consistent with the reported symptoms and the force involved in the collision. Examination revealed some findings supportive of a concussive injury, including mild balance deficits and reduced performance on portions of cognitive screening, while other neurological tests, including cranial nerve function, strength, and coordination, remained within normal limits. No prior injury to the head or neck region is documented in the available history. Treatment frequency decreased steadily as symptoms improved, and the patient was discharged from active care once symptoms had largely resolved. Billed services reflect the treatment provided, with no duplicate or repeated charges identified.